

POLICY BRIEF

YOUTH MENTAL HEALTH IN KENYA

From Listening to Implementation

A youth-led framework for accessible, affordable and youth-friendly mental-health support to address Kenya's growing crisis.

AUGUST 2026

CORE PROPOSITION

Kenya should move mental-health support closer to where young people live, learn, work and socialise, while making care affordable, confidential, non-judgmental and connected to specialist support.

ABOUT POLICYBRIDGE KE

PolicyBridge KE is a youth-led public policy and civic engagement organisation that creates meaningful pathways for young people to shape public policy through participation, dialogue and solution-building. Our approach combines youth consultation, community dialogue and solution-building. Through national consultations, community engagements and the Policy Lab, we generate both quantitative and qualitative insights on emerging policy issues and translate youth-generated ideas into evidence-informed policy proposals.

EXECUTIVE SUMMARY

The policy problem: Kenya has an established policy and legal basis for mental-health reform, including a statutory direction toward community-based care and mental-health services in primary health facilities. Yet access remains a central implementation concern. The National Youth Consultation conducted by PolicyBridge KE between **16 July** and **1 August 2026** across **20 Counties** recorded an average access-to-affordable-services score of only **2.87/10**; 51 of 78 respondents (**65.4%**) rated access at **1-3/10**.

What young people told us: Economic insecurity was the dominant reported pressure: financial stress was selected by 61 respondents (**78.2%**) and unemployment by 58 (**74.4%**). Social media (26), relationship challenges (23) and family conflict (20) were also prominent. While these findings do not establish causation, they show that participants understand mental well-being as closely connected to livelihoods, education, relationships and digital environments.

What young people prioritised: Mental-health education in schools was the most selected government priority (**44** respondents), followed by youth-friendly services (**37**) and more community counselling centres (**29**). Safe youth spaces (**21**), digital counselling platforms (**19**) and peer-support programmes (**18**) also featured prominently. The relatively low mention of specialist services may signal that young people perceive specialist care as less accessible or less relevant as a first point of contact (it should not be read as evidence that specialist care is unnecessary).

What the solutions add: The Policy Lab submissions translate these priorities into practical interventions: ward-level integration of basic mental-health support; institutional access points in schools, TVETs and universities; peer-led and community-based support; Mtaani Skill & Vibe Hubs; and a Creative Prescription Programme. PolicyBridge KE synthesises these proposals into a **four-pillar** framework rather than presenting them as competing interventions.

THE POLICY ASK

Kenya should move from a predominantly specialist, facility-centred model towards a connected youth mental-health system: prevent and identify distress early; provide basic support close to home and within institutions; create credible referral pathways; use digital tools to extend human care; and make financing and performance visible.

KEY NUMBERS

78	20	2.87/10
consultation responses	counties represented	mean access score

ACKNOWLEDGMENTS

PolicyBridge KE acknowledges the young people who participated in the National Youth Consultation on Mental Health in Kenya, the Policy Labs innovators who submitted policy solutions, and the community members who participated in dialogue and voting. Their experiences, recommendations and practical ideas form the centre of this brief.

PolicyBridge KE acknowledges Ms Sharon Gatugi Gikundi and Mr Kenson Mutethia for their analytical and editorial leadership in developing the policy challenge, reviewing the consultation responses and shaping this policy brief.

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Every effort has been made to verify the accuracy of the information contained in this publication.

All information was believed to be correct as of **August 2026**. Nevertheless, PolicyBridge KE cannot accept responsibility for the consequences of its use for other purposes or in other contexts.

1. THE POLICY PROBLEM

Kenya has a policy direction on mental health. Article 43 and the Fourth Schedule of the [Constitution of Kenya](#) guarantee the right to the highest attainable standard of health. The [Mental Health Act](#), as amended in 2022, gives statutory recognition to a right to accessible and affordable mental-health services and prioritises community health and outpatient primary mental-health care. It also places specific responsibilities on county governments for mental-health services, community-based care, financing, personnel and stigma-reduction programmes (see sections 2A, 2B, 2D and 3A).

The implementation question is therefore central. Although the State's policy and legal framework points toward prevention, early intervention, community care and integration into primary health services, the youth evidence gathered by PolicyBridge KE suggests young people still experience the system as distant, costly, difficult to navigate and insufficiently youth-friendly.

The cost of inaction. The [Kenya Mental Health Investment Case 2021](#) estimated the annual economic burden of selected mental-health conditions at **KSh.62.2 billion**, including **KSh.56.6 billion** in indirect costs, and modelled **KSh.81.7 billion** in investment over ten years for selected clinical and population-based interventions, with **KSh.161.6 billion** in projected returns over the same period. These investment-case estimates establish the economic logic for acting early and at scale even though they are not a current-year budget requirement yet. They also make the policy conclusion clear: mental-health financing requires identifiable allocations, transparent expenditure reporting and stronger alignment between national policy and county implementation.

WHY ACTION IS URGENT

With this policy brief, Kenya now has three assets that should not be allowed to sit separately: an established legal and policy framework, [the National Baseline Mental Health Survey in 2025](#) and a youth-generated body of evidence identifying concrete access barriers and preferred entry points. [The Ministry of Health's Suicide Prevention Strategy 2021–2026](#) also provides an existing platform for integrating suicide prevention into the proposed youth access model. The opportunity is to convert these assets into funded implementation rather than another cycle of consultation without delivery.

THE IMPLEMENTATION GAP

The challenge is not simply to add more specialist services. That traditional route is disconnected from the young people. The challenge is to connect prevention, early identification, basic support, referral and specialist care across **the places young people already use**.

2. METHODOLOGY AND DATA

Three youth engagement methods were used, each with a distinct purpose. They were analysed separately, then considered together to develop the policy framework.

METHOD	PURPOSE	TREATMENT OF EVIDENCE
Policy Lab Submissions	Crowdsource practical, youth-led solutions to the Policy Challenge.	We screened submissions for relevance and completeness, excluding incomplete submissions. We reviewed qualifying proposals and four solutions were voted for policy synthesis.
Community Meetings	Capture young people's lived experiences, concerns and priorities through open discussion.	Discussion notes were reviewed and synthesised thematically. Findings are qualitative and contextual, not statistically representative.
National Youth Consultation	Gather quantitative and open-ended perspectives on mental health experiences, access to support and policy priorities.	We analysed 78 online responses from 20 counties using descriptive statistics and thematic coding of open-ended responses.

Ethical and Data Protection Considerations

Participation in the National Youth Consultation was voluntary and respondents were required to confirm their agreement to submit their responses. The analysis and presentation of the consultation findings are undertaken at an aggregate level, and no individual respondent is identified in this policy brief. The Community Meetings were conducted as participatory engagements, with consideration given to participant privacy and confidentiality. Individual contributions are not attributed to identifiable participants in this brief. Policy Lab Submissions are presented as policy proposals rather than as individual testimonies. Personal identifying information is not reproduced in the policy analysis. The collection, handling, and reporting of personal information follow the [Data Protection Act, 2019](#), including its requirements for lawful and responsible processing of personal data.

National Youth Consultation

The online consultation ran from **16 July to 1 August 2026** and received **78** responses from participants reporting residence in **20 counties**. The questionnaire covered perceived drivers of mental-health challenges, access to affordable services, barriers to seeking support, help-seeking channels, government priorities and recommendations for policy action. We analysed closed-ended questions using frequencies and percentages. Multiple-response questions are non-exclusive, so percentages may exceed 100%. Open-ended responses were thematically reviewed.

Interpretation and limitations

- 1 The consultation is a rapid policy listening exercise, not a prevalence survey and not a statistically representative sample of Kenya's youth population.
- 2 Participation was voluntary and online, creating potential self-selection and digital-access bias.
- 3 Self-reported responses reflect participants' perceptions and experiences; they do not constitute clinical diagnoses.
- 4 Thematic coding of open-text responses identifies policy signals. It is not a validated clinical coding framework.
- 5 Platform submissions are self-selected proposals and should not be interpreted as a quantitative ranking of youth preferences.

WHO PARTICIPATED IN THE CONSULTATION

The consultation received **78** responses from participants in **20** counties. The sample was predominantly young adults aged **18–30**, and students and unemployed respondents together accounted for 55 participants (**70.5%**). Nairobi City accounted for 31 responses (**39.7%**), with the remaining responses distributed across 19 other counties. The consultation provides a particularly strong signal from young people navigating education and labour-market transitions, but it remains too small and self-selected to represent all Kenyan youth.

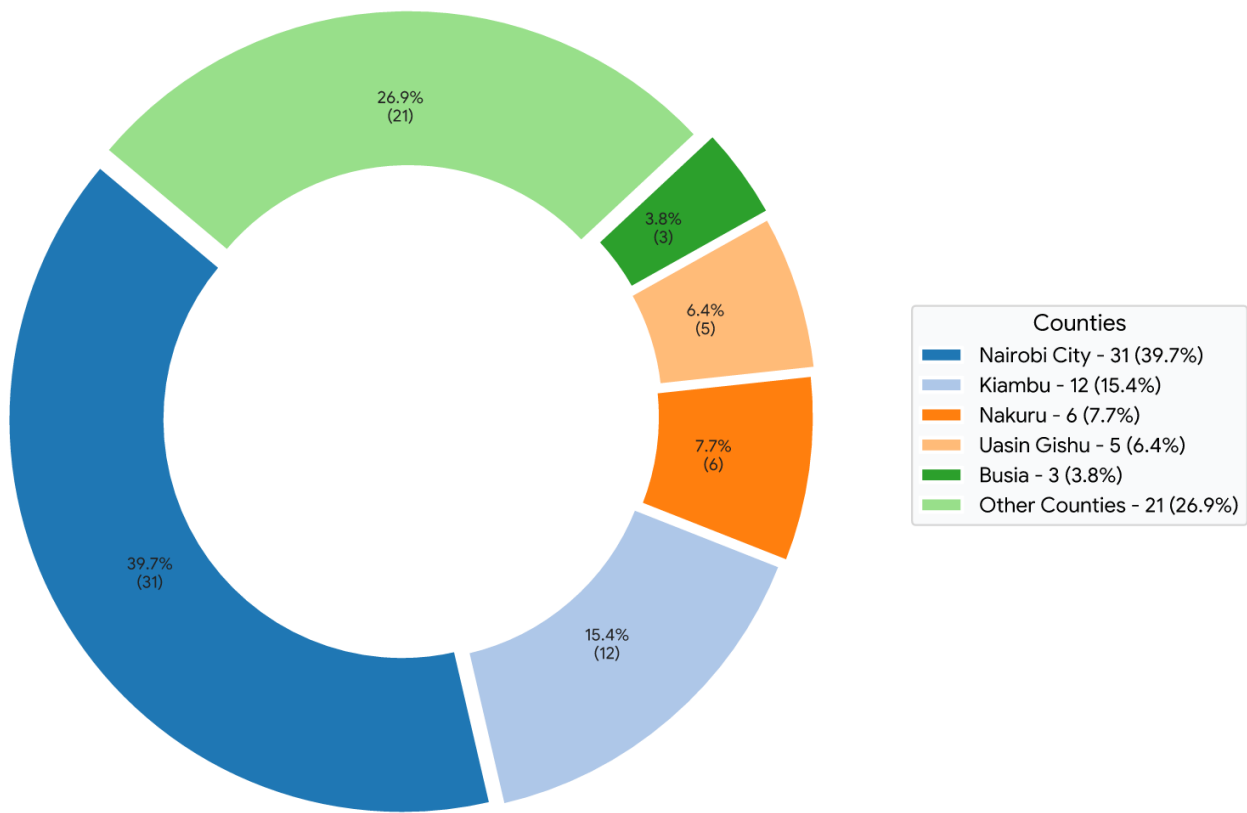
DATA EQUITY AND FUTURE DISAGGREGATION

While this consultation provides a useful national policy signal, it does not support robust subgroup analysis by sex, disability, county type or other characteristics. Future rounds may deliberately include groups that may face additional access barriers and, where ethically and operationally appropriate, collect disaggregated data on sex, disability, location, age, beliefs and institutional status. Service design may also provide reasonable accessibility and culturally appropriate pathways for young people who do not use mainstream services.

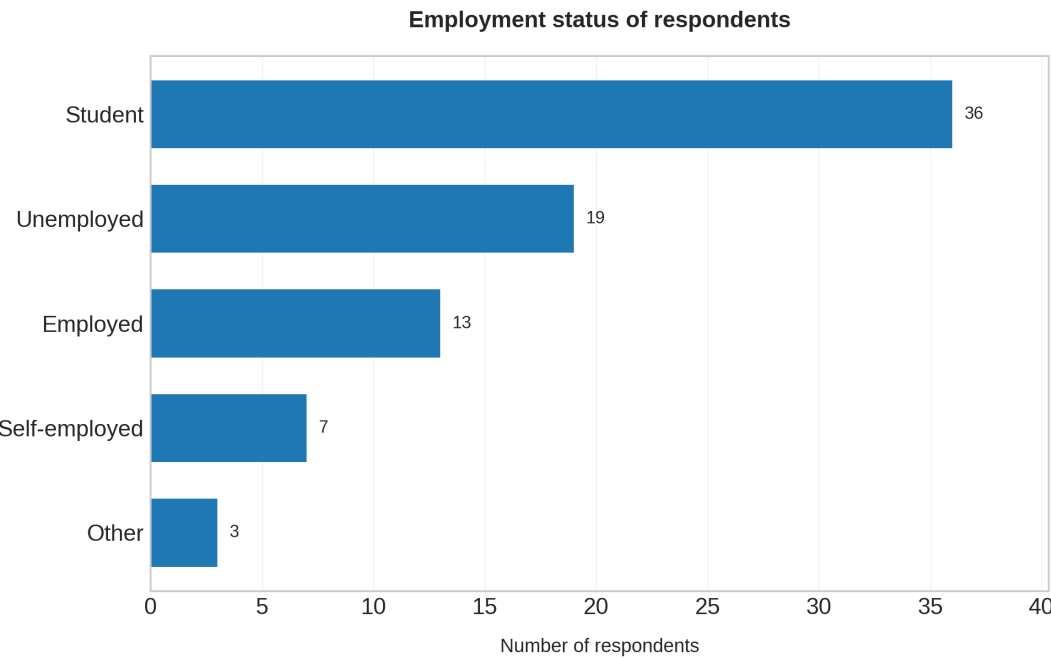
PROFILE	RESPONDENTS	SHARE
18–24	39	50.0%
25–30	22	28.2%
31–35	9	11.5%
Above 35	8	10.3%
Student	36	46.2%

PROFILE	RESPONDENTS	SHARE
Unemployed	19	24.4%
Employed	13	16.7%
Self-employed	7	9.0%

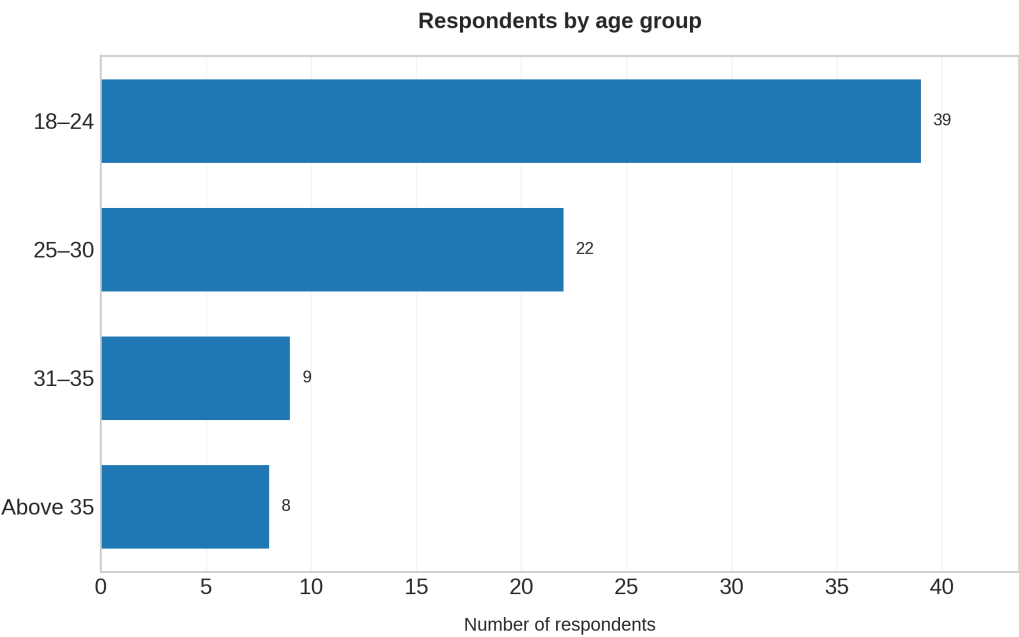
Counties Where Respondents Come From



Source: PolicyBridge KE National Youth Consultation, 2026.



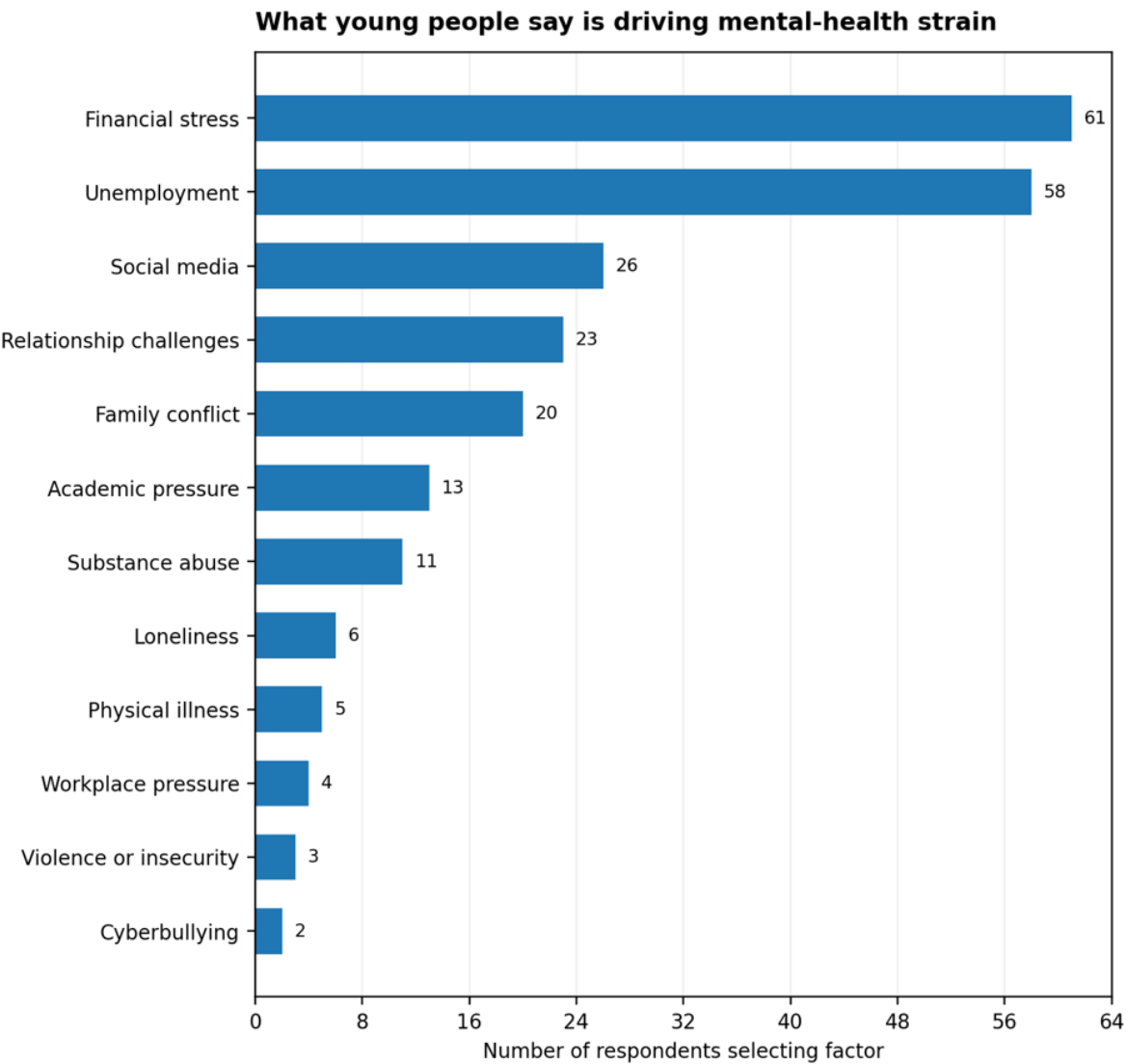
Source: PolicyBridge KE National Youth Consultation, 2026. Multiple responses permitted.



Source: PolicyBridge KE National Youth Consultation, 2026. Multiple responses permitted.

3. WHAT YOUNG PEOPLE TOLD US

The consultation points to a three-part problem: the pressures affecting well-being are strongly socio-economic and relational; perceived access to affordable care is low; and young people favour prevention, proximity and youth-friendly entry points.

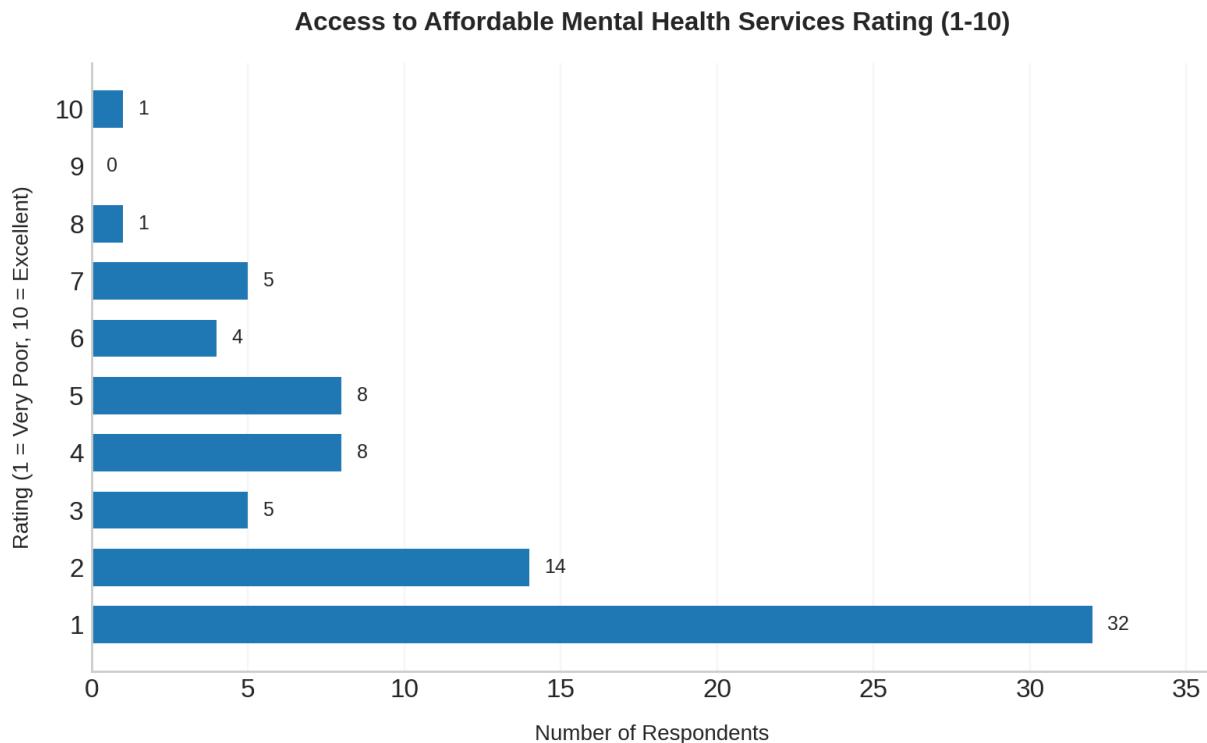


Source: PolicyBridge KE National Youth Consultation, 2026. Multiple responses permitted.

Economic pressures are central to the consultation. 61 respondents (**78.2%**) selected financial stress, and 58 (**74.4%**) selected unemployment. Social media (26), relationship challenges (23) and family conflict (20) were also frequently selected. While these responses do not establish causation or diagnose mental-health conditions, they do, however, indicate that young participants experience mental well-being closely linked to livelihoods, relationships, education and digital environments.

ACCESS IS PERCEIVED AS WEAK

The mean rating for access to affordable mental-health services was **2.87** out of **10**. Fifty-one respondents (**65.4%**) rated access between 1 and 3, including 32 who gave the lowest rating of 1. Only seven respondents (**9.0%**) rated access at 7 or above, and one respondent rated it 10. The distribution suggests that participants **strongly perceive** affordable, accessible services as difficult to obtain.



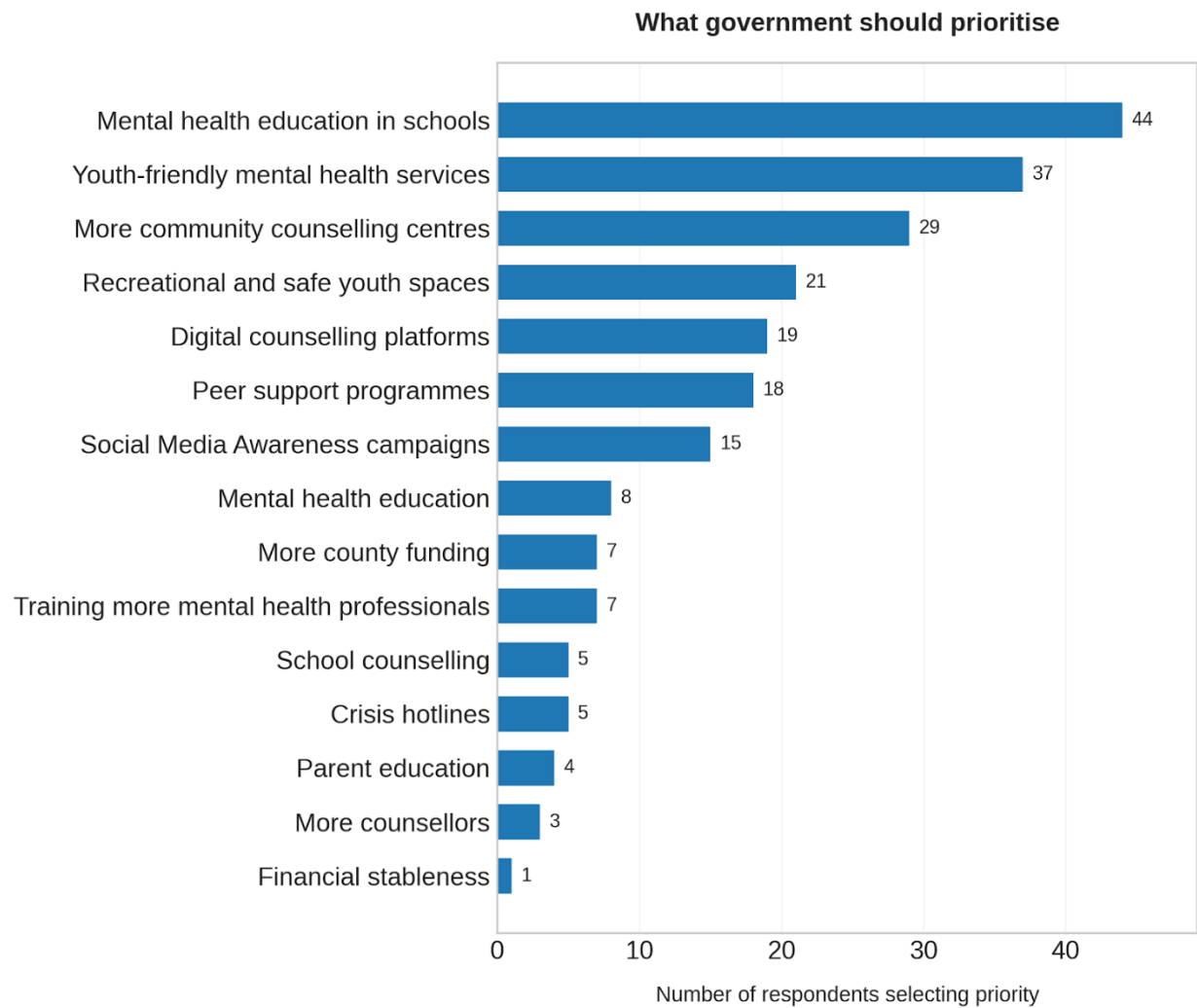
Source: PolicyBridge KE National Youth Consultation, 2026.

WHAT THIS MEANS FOR POLICY

The consultation does not call for a single new programme. It points toward a connected access system: mental-health literacy and prevention; accessible community and primary-care support; trusted peer and institutional entry points; and referral pathways to qualified professionals.

4. WHERE YOUNG PEOPLE WANT GOVERNMENT TO ACT

The strongest government priorities were mental-health education in schools (**44**), youth-friendly services (**37**), more community counselling centres (**29**), safe youth spaces (**21**), digital counselling platforms (**19**) and peer support programmes (**18**).



Source: PolicyBridge KE National Youth Consultation, 2026. Multiple responses permitted

The priority pattern: The leading preference, mental-health education in schools, is **preventive**. The next priorities, youth-friendly services and community counselling centres, focus on **proximity** and access. Safe spaces, digital counselling and peer support point to demand for **lower-stigma routes** into **support**.

PolicyBridge KE asked : If you could recommend ONE policy to improve youth mental health in Kenya, what would it be?

Sample Responses ...

YOUTH 1

Most of the youths have ways of venting drugs, screen and parting being top of the list. Focus should be creating awareness

YOUTH 2

Come up with a youth advisory council to advise on youths mental health.

YOUTH 3

Integration of mental health in the education, health and workplaces

YOUTH 4

There's no reason to assume someone is lying when they say they are not okay

YOUTH 5

Suicide rates among the youth are rising and this is not a small problem.

YOUTH 6

There is no health without mental health. Every policy affecting education, employment, housing, or technology should consider its impact on young people's mental well-being.

YOUTH 7

There are alot of Youth going through a lot, but you can't know because they are in the houses, they trust no one. And the best way to handle this is to visit communities .

YOUTH 8

Youth are not represented in the counselor and psychologist board. Youth are paying for licenses and zero employment opportunities for psychologists. We are having a brain drain to other careers.

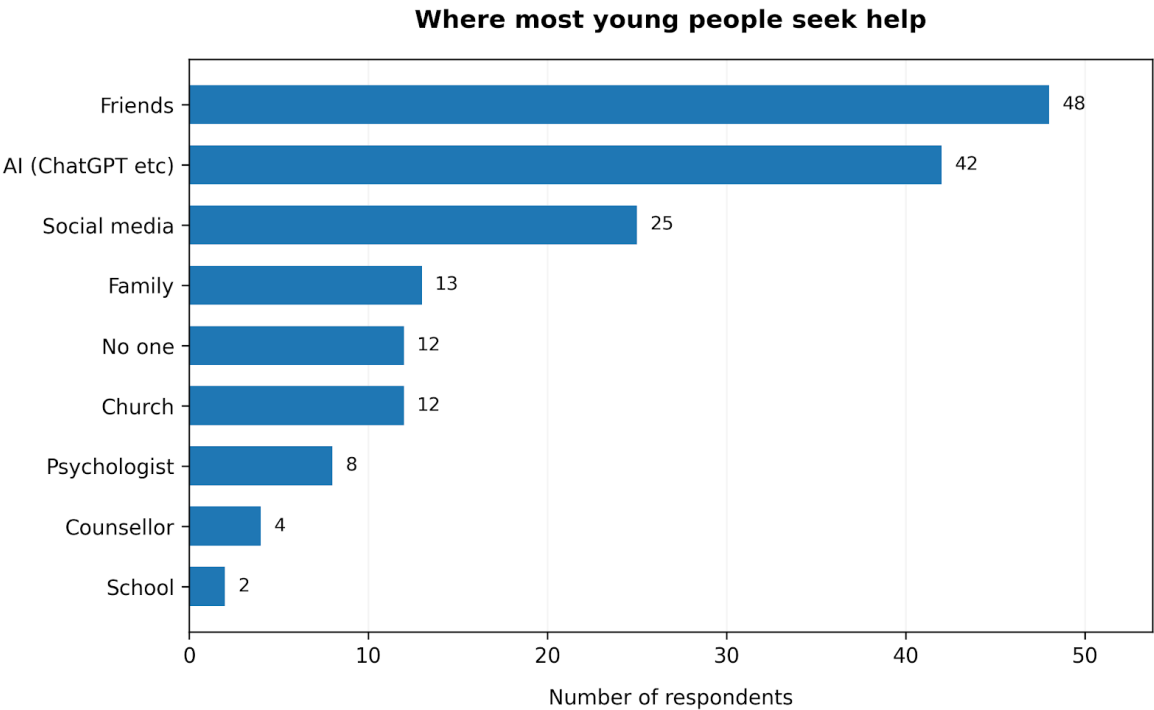
Source: PolicyBridge KE National Youth Consultation, 2025.

5. WHERE YOUNG PEOPLE SEEK HELP

Help-seeking is already distributed across informal and digital channels.

In the consultation's multiple-response help-seeking question, respondents mentioned friends (48), AI tools (42), social media (25), family (13), church (12) and psychologists (8). The responses can be interpreted as below.

S/NO	SOURCE	FREQUENCY
1	Friends	Very high
2	AI (ChatGPT, etc.)	Very high
3	Social media	High
4	Family	Moderate
5	Church	Moderate
6	No one	Notable



Source: PolicyBridge KE National Youth Consultation, 2026. Multiple responses permitted

While these figures reflect channels mentioned by respondents, not exclusive preferences or measures of effectiveness, the policy implication is that [informal and digital channels are already part of the youth support environment](#) and should connect to safe, confidential, and professionally supervised referral pathways.

AI, DIGITAL SAFETY AND CRISIS ACCESS

The finding that 42 respondents mentioned AI tools as a help-seeking channel is a material policy signal. Digital tools can widen access, but they can also produce unsafe or misleading guidance, create false reassurance in crisis situations, or expose sensitive health information. We already have a legal architecture governing health data and digital health, including the [Data Protection Act 2019](#) and [Digital Health Act 2023](#). Any public or publicly supported digital mental-health pathway should therefore apply data minimisation, appropriate consent and confidentiality safeguards, secure storage and transmission, clear provider accountability and human oversight.

Pertinently, AI should be treated as an **access and navigation adjunct**, not as an autonomous clinical decision-maker. Where a digital interaction indicates possible imminent self-harm or suicide risk, the system should provide a clear crisis response pathway and connect the user to qualified human support or emergency services. Public communication should make the limits of AI explicit, and any government-supported platform should be tested for safety, bias, escalation performance and data protection before scale-up. [Kenya's AI Strategy 2025–2030](#) provides a broader policy context for responsible AI deployment.

6. BARRIERS TO HELP-SEEKING

Open-ended responses repeatedly pointed to four interconnected barriers: **stigma** and fear of judgement; **limited awareness** of where and how to obtain support; **cost and affordability**; and concerns about **trust, confidentiality and the quality or availability** of services. The responses should be read as qualitative signals rather than a statistically ranked list. Together, they reinforce the case for services that are [confidential, affordable, easy to navigate and located](#) in settings that young people already trust.

DESIGN PRINCIPLE

Youth-friendly care must reduce the social and practical cost of asking for help: it should be confidential, affordable, non-judgmental, easy to find and connected to a qualified human referral pathway.

7. MENTAL HEALTH AND ECONOMIC SECURITY

The consultation also points to a policy issue beyond the health sector. Financial stress and unemployment were the two most frequently selected pressures. Mental-health policy should therefore be **coordinated** with youth employment, skills, enterprise development and social protection policies. This does not mean treating economic insecurity as a clinical condition; it means recognising social and economic conditions as part of the policy environment that produces and protects mental well-being. We believe existing youth employment, digital-skills, enterprise, and social-protection programmes such as [the Youth Enterprise Development Fund](#), [the Uwezo Fund](#), [the Hustler Fund](#), [the Women Enterprise Fund](#), and [the National Youth Opportunities Towards Advancement \(NYOTA\)](#) project should incorporate basic mental-health literacy, referral, and safeguarding, rather than creating separate parallel programmes. Our mental-health legal framework itself calls for action on determinants and

cross-sectoral prevention, while [WHO guidance](#) supports collaboration across health, education, employment and social-protection systems.

CROSS-SECTOR PRIORITY	POLICY DIRECTION
Youth employment and livelihoods	Strengthen access to employment, apprenticeships, skills and decent-work pathways, particularly for young people transitioning from education into the labour market.
Youth enterprise and financial inclusion	Improve access to affordable and responsible finance, business support and market opportunities, building on PolicyBridge KE's Youth Access to Capital Five Pillar Framework .
Social protection	Ensure vulnerable and unemployed young people can access appropriate social-protection and referral mechanisms without stigma or unnecessary administrative barriers.
Workplace and livelihood settings	Extend mental-health literacy, prevention and referral mechanisms to workplaces and youth enterprise programmes, consistent with existing workplace mental-wellness guidance.

8. THE YOUTH MENTAL HEALTH ACCESS AND PREVENTION FRAMEWORK

PolicyBridge KE proposes four mutually reinforcing pillars. The framework synthesises the National Youth Consultation, Community Meetings and the four selected youth-generated Policy Lab solutions. It is designed to **strengthen rather than duplicate** Kenya's existing health, education, and community systems.

PILLAR I: BRING MENTAL HEALTH TO THE WARD

Policy action: Integrate basic mental-health screening, psychological first aid, brief evidence-informed support and referral into primary healthcare; train CHPs, nurses and other frontline workers to [defined competencies](#); link facilities to county specialists; and use youth peer champions for navigation and follow-up. Frontline workers should identify, support and refer within protocol, not independently diagnose or manage complex or high-risk conditions. Routine primary-care pathways should include [suicide-risk identification](#), immediate safety planning within scope, escalation to qualified clinical care and documented follow-up for people identified as at risk.

Institutional responsibility: **Lead:** Ministry of Health and County Departments of Health. **Support:** professional bodies, youth organisations and development partners.

Accountability indicator: **Core measure:** proportion of participating primary-care facilities offering defined mental-health screening/support and functioning referral pathways.

PILLAR II: MAKE EDUCATION AND TRAINING INSTITUTIONS ACCESS POINTS

Policy action. Establish or strengthen mental-health desks in secondary schools, TVETs and universities; provide mental-health literacy and help-seeking information; train peer educators within safeguarding frameworks; and establish referral links to health services.

Institutional responsibility. **Lead:** Ministry of Education, training institutions and County education structures, in coordination with health authorities.

Accountability indicator. **Core measure:** institutions with functioning support, referral and safeguarding arrangements, not merely designated desks.

PILLAR III: BUILD COMMUNITY SAFE SPACES AND CREATIVE PREVENTION

Policy action: Pilot Mtaani Skill & Vibe Hubs in selected wards, using four tracks: Creative; Skill & Hustle; Movement & Body; and Social. Pilot a Creative Prescription Programme through which appropriate referrals can lead to structured, time-bound creative projects with professional check-ins. The model should be presented as a prevention, connection and social-prescribing intervention, not a substitute for clinical treatment. [WHO Manila Toolkit 2022](#) and [WHO's Report 67, 2019](#) support the potential contribution of arts and community-based social prescribing to health and well-being, while also emphasising the need for context-specific evaluation

Institutional responsibility: **Lead:** County Governments and youth-led/CSO partners. **Support:** local creatives, universities and responsible private-sector/CSR partners.

Accountability indicator: **Core measure:** participation, retention, referrals completed, participant-reported well-being and connection outcomes, safeguarding incidents, and cost per participant retained. Pilots should include a simple evaluation design before scale-up.

PILLAR IV: CONNECT DIGITAL, CRISIS AND SPECIALIST SUPPORT

Policy action: Strengthen an integrated digital access and referral layer using web, SMS and messaging channels, with confidential triage, clear escalation protocols and referral to qualified human support. Digital and AI tools should supplement, not replace, professional care. The model should include: clear crisis escalation for suicide and self-harm risk; human review for high-risk cases; public disclosure of AI limitations; data-minimisation, consent and security controls; audit and incident-reporting mechanisms; and low-bandwidth and non-smartphone pathways. Digital systems should align with the Digital Health Act, Data Protection Act and applicable health standards.

Institutional responsibility: **Lead:** Ministry of Health and relevant digital-health institutions, with county and specialist-provider linkages.

Accountability indicator: **Core measure:** successful connection to qualified support; referral completion; response time for urgent cases; percentage of high-risk interactions escalated to human support; documented privacy or safety incidents; and user-reported trust, confidentiality and accessibility.

COUNTY ADAPTATION AND PILOT SELECTION

We appreciate that given the different fiscal capabilities and priorities of our devolved units, the four pillars may not be implemented as a uniform national template. With this in mind, we propose that counties adapt the **service mix to context** while retaining a **common minimum package**, safeguarding

standard, referral protocol and measurement framework. At minimum, implementation should recognise urban/metropolitan, rural/agricultural, arid and semi-arid or pastoralist, and fragile or conflict-affected settings. Adaptation may include balancing facility and outreach services, language and communication methods, mobile or community delivery, school and TVET reach, transport and referral arrangements, and the role of trusted community institutions.

Pilot counties should be selected against a transparent criteria: (1) unmet need and access barriers; (2) county leadership commitment and budget readiness; (3) minimum health-system and referral capacity, or a credible plan to establish it; (4) geographic and socio-economic diversity; (5) availability of youth and community partners; and (6) capacity to collect and report agreed indicators. The pilot portfolio should deliberately include different county contexts rather than selecting only counties with strong existing infrastructure.

9. FINANCING, ACCOUNTABILITY AND YOUTH GOVERNANCE

Implementation will fail if mental health remains visible in policy documents but difficult to identify in budgets and performance reports. Financing should therefore be part of implementation design, not a later fundraising exercise. The [Kenya Mental Health Investment Case 2021](#) estimated an annual economic burden of **KSh 62.2 billion** in **2021** and found returns of **KSh 4** for every **KSh 1** invested in scaled-up depression interventions and **KSh 2.30** for anxiety interventions. These figures strengthen the economic case for investment, although they do not reflect current costing under the PolicyBridge KE framework.

This brief does not currently present project costs. As such, we propose that, before national or county adoption, the Ministry of Health, National Treasury, and county governments prepare a costed implementation plan using actual unit costs. The plan should cost workforce training and supervision, youth participation, digital and crisis infrastructure, school and community components, monitoring and evaluation and recurrent operating costs separately. PolicyBridge KE's complementary costed implementation annex is available upon request.

FINANCING LEVER	POLICYBRIDGE KE PROPOSAL
National and County Budget Lines	Create identifiable mental-health budget lines linked to measurable outputs and publish allocations and expenditure.
Primary Healthcare and Health Financing	Ensure eligible mental-health services are incorporated into applicable primary-care and health-financing arrangements, with tariffs and referral pathways that do not make care unaffordable.
Development Partners and CSR	Use external and private financing for catalytic pilots, infrastructure and innovation, with a defined transition plan into public financing.

Performance-linked Accountability	Tie continued scale-up to agreed coverage, referral, quality and expenditure indicators rather than announcements alone.
REFORM	WHAT SHOULD CHANGE
Make mental-health expenditure visible	National and county health plans and budgets should clearly identify mental-health allocations and programmes so they can be tracked from allocation to expenditure and service delivery.
Publish implementation data	Annual public reporting should include selected indicators such as facilities offering services, trained frontline workers, youth uptake, referral completion and waiting times.
Link financing to service standards	Funding should support defined packages of prevention, early identification, basic support and referral rather than only infrastructure or specialist institutions.
Strengthen youth accountability	Formalise meaningful youth participation in programme design, monitoring and evaluation, including paid participation where young people provide substantive expertise.

10. YOUTH GOVERNANCE AND ACCOUNTABILITY

PolicyBridge KE **proposes institutionalising youth participation rather than limiting** it to consultation. We recommend a standing [Youth Mental Health Advisory](#) mechanism at national and county levels (Nairobi County already has [the Nairobi Youth Advisory Council](#)), with transparent terms of reference and meaningful participation in programme design, budgeting priorities, monitoring and evaluation. Participation should include rural and marginalised youth and should be appropriately supported or compensated when young people provide substantive policy or technical input.

11. IMPLEMENTATION ROADMAP

REFORM AREA	0-6 MONTHS	6-18 MONTHS	18-36 MONTHS
Primary care	By month 3: issue minimum package, screening/referral tools and pilot criteria.	Months 6-12: train and supervise frontline workers; activate referral links. Months 12-18: review competency and referral performance.	By month 24: scale only where outcome and cost data support expansion; integrate into county plans by month 36.
Education	By month 3: issue minimum standards, safeguarding and referral guidance.	Months 6-12: implement in pilot institutions; months 12-18: measure awareness, use and referral completion.	By month 36: integrate effective indicators into routine education and health oversight.
Community & Creative Hubs	By month 4: map existing spaces and select pilot hubs; approve safeguarding model.	Months 6-12: launch hubs and creative/social-prescribing pilots; months 12-18: evaluate retention and outcomes.	By month 36: scale only models meeting agreed effectiveness, safety and cost thresholds.
Digital & referral	By month 3: map existing digital and crisis assets; approve governance and escalation protocols.	Months 6-12: pilot integrated access; months 12-18: audit safety, privacy and referral completion.	By month 36: expand interoperable, low-bandwidth services that meet safety and performance standards.
Finance & accountability	By month 3: create identifiable budget/reporting lines and baseline indicators.	By month 12: publish allocation and expenditure data for pilots; by month 18: publish implementation results.	Annual public scorecard, independent/participatory review and rolling cost update.

12. IMPLEMENTATION RISKS AND MITIGATION

The framework is deliberately phased because implementation risks are material. The principal risks and practical mitigations are:

RISK	LIKELIHOOD	IMPACT	MITIGATION	RISK OWNER
Financing shortfalls	High	High	Identifiable budget lines; expenditure tracking; phased pilots linked to evidence of results.	National Treasury/ Ministry of Health (MoH)/counties
Workforce constraints	High	High	Task-sharing within defined scopes; supervision; training pipelines; specialist referral and tele-mental-health links.	MoH/county health departments/professional bodies
Stigma and low uptake	High	High	Peer support; community dialogue; confidential service design; sustained public education.	MoH /Education/counties/youth partners
Digital exclusion	Medium	Medium	SMS/USSD and other low-bandwidth options; community access points; avoid digital-only delivery.	County governments/partners
Data protection and confidentiality	Medium	High	Data-minimisation; consent; role-based access; secure systems; clear clinical escalation and governance protocols.	MoH/Digital Health Agency/ODPC
Institutional coordination	Medium	High	Named responsibilities; intergovernmental coordination; common indicators and periodic public reporting.	MoH/county coordination structures

13. POLICY, LEGAL AND INSTITUTIONAL ALIGNMENT

The framework is intentionally implementation-oriented. It does not propose a parallel mental-health system. It builds on Kenya's constitutional right to health, the [Mental Health Act](#) as amended in 2022, the [Health Act](#), the [Kenya Mental Health Policy 2015–2030](#) and the primary-healthcare and digital-health architecture. The [Kenya Mental Health Action Plan 2021–2025](#) remains an important implementation reference, but its stated implementation period has ended; the immediate policy question is therefore how its relevant commitments are renewed, operationalised and monitored beyond **2025**. The Ministry of Health also published [the National Baseline Mental Health Survey in 2025](#), providing an important national evidence base against which future youth-focused monitoring can be aligned.

POLICY WINDOW

Kenya now has both a strengthened statutory framework and a national baseline survey. The opportunity is to translate those assets into measurable, decentralised and youth-friendly implementation.

FRAMEWORK	RELEVANCE TO THIS BRIEF
Constitution of Kenya , Article 43	Right to the highest attainable standard of health and access to healthcare services.
Mental Health Act as amended in 2022	Accessible and affordable mental-health services; priority to community health and outpatient primary mental-health care; defined county responsibilities.
Health Act , Cap. 241	National/county health-system architecture and progressive access to promotive, preventive, curative, palliative and rehabilitative services.
Kenya Mental Health Policy 2015–2030	National framework for mental-health system reform.
Kenya Mental Health Action Plan 2021–2025	Implementation framework addressing governance, financing, service access, prevention and systems strengthening.
Primary Health Care Act 2023 and Digital Health Act 2023	Relevant statutory architecture for primary-care delivery and digital-health services. Governance, confidentiality, privacy and security requirements relevant to digital mental-health services and health data.

Kenya Suicide Prevention Strategy 2021–2026	Existing national framework for suicide prevention, surveillance, awareness, integrated interventions and multisectoral action.
Kenya AI Strategy 2025–2030	Broader national policy context for responsible AI deployment, skills, innovation and governance.

INSTITUTIONAL RESPONSIBILITIES

ACTOR	PRIORITY RESPONSIBILITY
Ministry of Health	Set standards and guidance; coordinate national monitoring; support workforce development; establish referral and digital-health governance.
County Governments	Implement ward-level services; allocate resources; support community programmes; monitor local performance.
Ministry of Education and institutions	Provide institutional support, literacy, safeguarding and referral arrangements.
National Treasury and county finance structures	Make mental-health allocations and expenditure sufficiently identifiable for accountability.
Youth-led organisations and CSOs	Co-design services; deliver peer/community components within appropriate standards; support engagement and monitoring.
Young people	Participate meaningfully in design, implementation, monitoring and accountability.

14. WHAT SHOULD BE MEASURED

A youth mental-health strategy should measure access and outcomes, not merely activity.

PolicyBridge KE recommends a compact public scorecard built around five questions:

- 1 **Reach:** How many primary facilities, schools, TVETs, universities and community hubs offer defined youth-friendly support?
- 2 **Use:** How many young people use those services, disaggregated where appropriate by age, sex, location and institution?
- 3 **Continuity:** What proportion of referrals are completed, and how quickly do young people move between levels of care?
- 4 **Experience:** Do young people report that services are confidential, respectful, affordable and non-judgmental?

- 5 **Resources:** What was allocated, what was spent, where was it spent, and what service coverage resulted?

A PRACTICAL ACCOUNTABILITY TEST

A programme should not be judged successful merely because a facility was designated, a training was conducted, or a budget was approved. Success should be demonstrated by whether young people can reach, use and benefit from appropriate support.

15. CONCLUSION: LISTENING MUST BECOME IMPLEMENTATION

The youth evidence points to a practical direction for reform. Participants identified economic insecurity, social pressures and weak access to affordable support as major concerns. They prioritised mental-health education, youth-friendly services, community counselling, safe spaces, digital access and peer support. The Policy Lab submissions then translated these concerns into practical models that can be piloted and connected to existing systems.

The policy response should therefore be connected and close to people. Mental-health support should be available through primary healthcare, educational institutions, trusted community settings and safe digital pathways, with clear referral to specialist and crisis care when required. Economic security should be treated as a cross-sector determinant, while youth participation should be built into design, budgeting, implementation and evaluation.

PolicyBridge KE's central proposition is straightforward: Kenya should build a youth mental-health system in which a young person can recognise a need for support, find a credible entry point close to where they live, learn or work, receive appropriate early assistance, obtain a safe response in a crisis, and move smoothly to higher levels of care when necessary. The evidence gathered through this youth-led process provides a practical starting point for that transition from policy commitment to implementation.

THE FIVE IMMEDIATE PRIORITIES

- 1 Integrate basic mental-health support and referral into primary healthcare.
- 2 Make schools, TVETs and universities active, safe access points.
- 3 Pilot community safe spaces and Creative Prescription pathways.
- 4 Connect digital and peer support to qualified human care and referral.
- 5 Align mental-health action with youth employment, livelihoods and social protection.
- 6 Make financing, performance and youth participation publicly traceable.

SOURCE NOTES AND REFERENCES

Primary PolicyBridge KE sources

- 1 PolicyBridge KE, National Youth Consultation on Mental Health in Kenya: questionnaire dataset, 78 responses, **16 July–1 August 2026**.
- 2 PolicyBridge KE 2026 Policy Challenge II: Youth-Led Solutions for Kenya's Mental Health Crisis Policy Lab Solutions.

National Policy and Legal Sources

- 3 [Constitution of Kenya](#), Article 43 and Fourth Schedule.
- 4 [Mental Health Act](#) as amended by the Mental Health (Amendment) Act, 2022, including sections 2A, 2B, 2D and 3A.
- 5 [Health Act](#), Cap. 241
- 6 [Primary Health Care Act 2023](#)
- 7 [Digital Health Act 2023](#)
- 8 [Kenya Mental Health Policy 2015–2030](#) (Ministry of Health)
- 9 [Kenya Mental Health Action Plan 2021–2025](#) (Ministry of Health)
- 10 [National Clinical Guidelines for Management of Common Mental Disorders, 2024](#). (Ministry of Health).
- 11 [The Ministry of Health's Suicide Prevention Strategy 2021–2026](#) (Ministry of Health).

Evidence and Financing context

- 12 [Kenya Mental Health Investment Case: The Case for Investment in Mental Health \(2021\)](#). (WHO / Government of Kenya).
- 13 [Mental Health Policy Brief, March 2023: Every Suicide Is a Preventable Tragedy, A Need for an Integrated Care Model in Kenya](#). (Ministry of Health).
- 14 [Mental Health Atlas 2024](#). (World Health Organisation).

National and International Implementation Guidance

- 15 [Ministry of Health, The National Baseline Mental Health Survey 2025](#).
- 16 [Mental Health Policy Brief, March 2023: Every Suicide Is a Preventable Tragedy, A Need for an Integrated Care Model in Kenya](#).
- 17 [Ministry of Information, Communications and the Digital Economy, Kenya's AI Strategy 2025–2030](#)
- 18 [World Health Organization, Guidance on Community Mental Health Services: Promoting Person-Centred and Rights-Based Approaches \(2021\)](#).
- 19 [World Health Organization, Peer Support Mental Health Services: Promoting Person-Centred and Rights-Based Approaches \(2021\)](#).
- 20 [World Health Organization, What is the evidence on the role of the arts in improving health and well-being? A scoping review \(2019\)](#).
- 21 [World Health Organization, A toolkit on how to implement social prescribing \(2022\)](#).

Methodological note: Consultation statistics in this brief are descriptive and calculated from PolicyBridge KE's dataset. Multiple-response questions are non-exclusive. Thematic coding is an analytical synthesis for policy purposes, not a clinical or epidemiological classification. The three engagement methods are kept analytically distinct: the National Youth Consultation provides the principal quantitative evidence; Community Meetings provide qualitative depth; and Policy Lab Submissions provide youth-generated policy proposals. External sources situate these findings within our legal, policy, and health-system context. Claims about national prevalence or causation are not inferred from the PolicyBridge KE consultation.